Suggested Initial ITU Management of Suspected COVID V5.2

Suggested management only. May be varied on advice from senior clinicians. Request urgent Senior/Intensivists review if parameter not achievable. Consider renal, hepatic and allergy status before prescribing		
If CPAP	Ensure Self-proning–ICS guideline Decision on intubation within 1 hour	Intubate if persistent hypoxia, low and falling P/F (eg <15-20), persistent high tidal volumes or high work of breathing
If ETT	Check ET cuff pressure/depth	Have ETT clamp at bedside
Initial ventilator settings **+** **First Recruitment Manoeuvre**	**Vent Setting** 1) Wet Circuit preferred. 2) Suggestion for initial SIMV: • FiO2 for SaO2>92-96%, • Vt 6ml/kg IBW, RR 24, I:E 1:1 • PEEP 5-8 (higher if poor compliance, oedema on CXR / recruitment effective) **Recruitment manoeuvre:** 1) Do not disconnect. 2) Fi02 to 0.8-1.0 +/- ↑ PEEP 3) Set VT to generate PAW 30-40 **4)** 'Inspiratory Hold' for 10-15s **5) RESET Vt to 6ml/kg**	**Targets:** pO2 >7.0, FiO2 <0.6, PEEP 8-15cmH_2O Max Vt 6/kg, Driving Pressure (=PAW-PEEP) ≤15 Any CO2, aim pH> 7.2 Consider echo if PEEP>10 or APRV (risk of acute cor pulmonale) **Options if targets not met at 4-6 hours:** 1) Prone 16-18hrs if P/F still <20, especially if CXR clear, normal compliance (>40ml/cmH_2O), 2) Paralysis infusion + BIS 3) APRV, especially if oedema on CXR, Poor compliance (<40ml/cmH_2O) or repeated recruitment . Fluid Bolus recommended. 4) RRT for mixed acidosis (consultant only) 5) Iloprost (intensivist only, may worsen hypoxia)
CVC, NGT, Catheter	CVC 16cm RIJV, 20cm LIJV (blood cultures from CVC)	No pH for NGT : X-Ray only, trained staff only Target urine output >0.5ml/kg/hr
Recovery Trial	In hours, contact research nurses on 4331 or Blp 181, or out of hours discuss with ICU consultant.	Ensure recruitment attempted, ensure trial drugs prescribed (see separate guidance)
		consider 2nd randomisation for Tocilizumab if CRP>75 and deteriorating
Covid Drugs	Dexamethasone	6.6mg OD x 10/7 IV
	Enoxaparin:	If <50Kg 20mg BD, 50-100Kg 40mg BD, 100-150Kg 60mg BD. If >150Kg or eGFR <30 seek senior/pharmacy adv
	Remdesivir	Do not stop if already started, do not start otherwise.
	Tocilizumab – must be given within 24hs	All ICU patients with proven COVID requiring respiratory support, unless contra-indicated. See 'request for supply of off label use of Tocilizumab'
	Monoclonal Antibodies (MAB)	Only as part of RECOVERY trial
Fluids and Feed	500ml CSL (max 2) if MAP <65 No maintenance fluid NG Feed once lactate normal	Tolerate positive fluid balance in 1st 24Hrs Consider FICE if fluid status unclear Standard ICU Feed protocol. If being proned, stop feed whilst turning.
Bloods / Investigations	U&E, LFT, Mg, Phos, Ca, CRP FBC, Coag, Trop T, D-Dimer, Ferritin, Procalcitonin, CK	If Troponin high or oliguric, request FICE Elevated Trop, D-Dimer and CRP>60 are markers of increased severity and worse prognosis
BP	Target MAP 65-70 (not higher) Noradrenaline first line If Norad >0.5mcg/kg/min request PICCO or Echo and inform senior	Consultant decision re 2nd agent Adrenaline/Dobutamine/Vasopressin Echo seeking cardiomyopathy

PLEASE TURN OVER

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Electrolyte Targets	K+ 4.0-5.0 Mg > 1.0 Phos > 0.7	40mmol KCL in 100ml, over 2 hours via CVC only 20mmol MgSO4 in 100ml NaCl 0.9%, 1 hour 20mmol Addiphos in 500ml Dextrose 5% / 24hrs
Microbiology	Resp Virus Swab, Covid Swab (NBAL 1st Choice vs Nose AND throat) NBAL, Blood Cultures, Urine culture + Urine for legionella/ pneumococcus PCR. Baseline procalcitonin, B-D-Glucan & galactomannon	Antibiotics only if ?bacterial co-infection (use Procalcitonin guide): unless Penicillin allergy Amoxicillin 1G TDS + Flucloxacillin 2g IV QDS + Clarythromycin 500mg NG BD
CPR status and communication	Clarify CPR status Update family when able	Consider DNACPR if Clinical Frailty Score 5 or more, severe co-morbidity, patient wishes.